

UUID: CA2DF1AC-137B-4A68-BAF5-66B16138F412
TCGA-EM-A3FN-01A-PR

Redacted



Surgical Pathology Consultation Report

Patient Name: [REDACTED] Accession #: [REDACTED]
MRN: [REDACTED] Service: [REDACTED]
DOB: [REDACTED] Visit #: [REDACTED]
Gender: [REDACTED] Location: [REDACTED]
Ordering MD: [REDACTED] Facility: [REDACTED]
Copy To: [REDACTED] Reported: [REDACTED]

1CD-0-3

Specimen(s) Received

1. Parathyroid: right lower parathyroid qs
2. total thyroidectomy stitch upper pole left lobe

Carcinoma, papillary, follicular variant

Site: thyroid, NOS C73.9

8340/3

hw 11/22/11

Diagnosis

1. No pathological diagnosis: Parathyroid (right lower) biopsy
2. Multifocal papillary carcinoma, dominant follicular variant, 4.2 cm, left; nonspecific mild thyroiditis: Thyroid
No pathological diagnosis: Lymph nodes, 2, isthmus
-Total thyroidectomy specimen

Synoptic Data

Procedure: Total thyroidectomy
Received: Fresh
Specimen Integrity: Intact
Specimen Size: Right lobe:
4.0 cm
2.3 cm
1.0 cm
Left lobe:
4.6 cm
3.2 cm
2.1 cm
Isthmus +/- pyramidal lobe:
1.6 cm
1.0 cm
0.6 cm
Specimen Weight: 25.6 g
Tumor Focality: Multifocal, ipsilateral
----- DOMINANT TUMOR -----
Tumor Laterality: Left lobe
Tumor Size: Greatest dimension: 4.2cm
Additional dimension: 3.1 cm
Additional dimension: 2.0 cm
Histologic Type: Papillary carcinoma, follicular variant

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer initials	hw 11/22/11	11/22/11

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Follicular architecture *pn-TSS, follicular variant = 100%*
Classical cytomorphology
Histologic Grade: Not applicable
Margins: Margins uninvolved by carcinoma
Tumor Capsule: Totally encapsulated
Tumor Capsular Invasion: Present, extent minimal
Lymph-Vascular Invasion: Not identified
Perineural Invasion: Not identified
Extrathyroidal Extension: Not identified
----- SECOND TUMOR -----
Tumor Laterality: Left lobe
Tumor Size: Greatest dimension: 0.05cm
Histologic Type: Papillary carcinoma, follicular variant
Papillary carcinoma, microcarcinoma (occult, small or microscopic) variant
Follicular architecture
Classical cytomorphology
Histologic Grade: Not applicable
Margins: Margins uninvolved by carcinoma
Tumor Capsule: None
Tumor Capsular Invasion: Not identified
Lymph-Vascular Invasion: Not identified
Perineural Invasion: Not identified
Extrathyroidal Extension: Not identified
TNM Descriptors: m (multiple primary tumors)
Primary Tumor (pT): pT3: Tumor more than 4 cm, limited to thyroid or with minimal extrathyroidal extension
(eg, extension to sternothyroid muscle or perithyroid soft tissues)
Regional Lymph Nodes (pN): pN0: No regional lymph node metastasis
Number of regional lymph nodes examined: 2
Number of regional lymph nodes involved: 0
Distant Metastasis (pM): Not applicable
Additional Pathologic Findings: Thyroiditis, focal (nonspecific)
Ancillary Studies: Type: IHC
Results: In the dominant nodule, cytokeratin 19 shows regional membranous positivity.
Stains for HBME1 and Galectin 3 are negative.

*Pathologic Staging is based on AJCC/UICC TNM.

Electronically verified by:

Gross Description

1. The specimen labeled with the patient's name and as "Right Lower Parathyroid QS" contains a piece of tissue that measures 0.3 x 0.2 x 0.1 cm. It is frozen for intraoperative consultation.

1A frozen tissue resubmitted

2. The specimen labeled with the patient's name and as "Total Thyroid Stitch Upper Pole Left Lobe Fresh Tumor Banking" consists of a thyroid gland that is oriented with a suture and weighs 25.6 g. The right lobe measures 4.0 cm SI x 2.3 cm ML x 1.0 cm AP, the left lobe measures 4.6 cm SI x 3.2 cm ML x 2.1 cm AP and the isthmus measures 1.6 cm SI x 1.0 cm ML x 0.6 cm AP. The external surfaces have fibrous adhesions and are painted with blue dye. No parathyroid glands or no lymph nodes are identified grossly. The left lobe contains well delineated nodule(s) that measure 4.2 cm SI x 3.1 cm ML x 2.0 cm AP. The remainder of the thyroid tissue is unremarkable. Tissue is stored frozen. The remainder of the specimen is submitted in toto as follows:

2A-H right lobe, superior to inferior

[REDACTED] **Surgical Pathology Consultation Report** [REDACTED]

2I-J isthmus
2K-R left lobe, superior to inferior

Quick Section Diagnosis

1. Parathyroid: right lower parathyroid QS: Parathyroid tissue identified.
Dr. [REDACTED] informed at [REDACTED]

Microscopic Description

In the dominant nodule, cytokeratin 19 shows regional membranous positivity. Stains for HBME-1 and galectin-3 are negative.
